



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : احمد معتوق على عماره

العنوان : ----

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 25601250101137 رقم التليفون : 01275929771

العمر (تاريخ الميلاد) : 25-01-1956 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه (-----) التوقيع: (-----)

التشخيص الإكلينيكي : سرطان القولون

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القولون

خطة العلاج : أبحاث أورام اشعه مقطعيه علي الصدر بالصبغه [10851] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ----- التوقيع : -----

الاسم : د/ندى حسن الوظيفة : ----- التوقيع : -----

الاسم : د/نرمين رمضان الوظيفة : ----- التوقيع : -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----

خطة العلاج : -----

المستشفى : ----- قسم : -----

جهة العلاج : ----- نفقات : -----

ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: احمد معتوق علي عماره National ID: 25601250101137	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

A patient of colon cancer for Contrast-enhanced chest CT
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL
TRANSFORMATION



الهيئة العامة للتأمين الصحي
الإدارة المركزية لنظم المعلومات و التحول
الرقمي

رسوم الاستعلام 20 جنية

جهة الاصدار: فرع القليوبية

تاريخ الاصدار: 2026-8-10

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للتأفك الخاضعين لقانون التأمين
الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات
والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 "
الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي

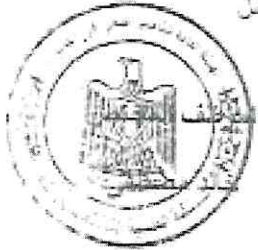
والبحث تبين أن

السيد (ة) احمد معنوق على بطاقة رقم 25601250101137

تليفون رقم: 01010349399

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المستفوعة بالتأمين الصحي صدر هذا الخطاب دون اننى مسئولية على الهيئة
العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة: 2026-8-10



إفادة مركز نظم المعلومات
بمكتبه...



سلسلة 2046824

الخطاب ساري لمدة ثلاثة اشهر من تاريخ الاصدار الا فرحله بغير حله الموافق من قبل الهيئة العامة للتأمينات الاجتماعية او
وزارة التأمين الاجتماعي او وزارة التربية والتعليم

الإدارة المركزية لنظم المعلومات و التحول الرقمي

جمهورية مصر العربية
بطاقة تحقيق الشخصية

أحمد
معتوق علي عماره
١٣ ش مسجل الرحمن
الخصوص - القنوية

١٩٥٦/٠١/٢٥ ٢٥٦.٠١٢٥.٠١.٠١١٣٧

K00069371

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٢٠٢٤/٠٣ ٢٥٦.٠١٢٥.٠١.٠١١٣٧
يكون عمل

معتوق مسلم ذكر

البطاقة سارية حتى ٢٠٣١/٠٣/٢٣ ١١



Name | Ahmed Maatouq Ali
Date | Wednesday, August 19, 2026

□ pelvi-abdominal U/S reveals

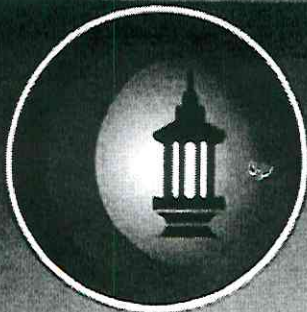
- Liver: is average in size with homogenous parenchymal echopattern. No intra-hepatic biliary radicle dilatation. Left lobe shows well defined iso to hypoechoic focal lesion measuring about 2.8 x 2.1 cm, taking no internal vascularity on applying color Doppler.
- PV & CBD (both are patent and not dilated).
- Gall bladder: is average in size shows biliary sludge with increased wall thickness (6mm). no stones
- Both kidneys: are within average size and normal site and shape. Normal parenchymal thickness with preserved cortico-medullary differentiation, yet shows increased parenchymal echogenicity bilaterally. No stones or back pressure changes on either side. Right kidney shows simple cortical cyst measuring about 1.2 x 0.8 cm.
- Rt kidney: 8.7 x 4.6 x 1.9 cm
- Lt kidney: 10.9 x 5 x 1.3 cm
- Spleen: is average in size with homogenous texture and no focal lesions. Splenic vein is not dilated.
- Pancreas and para-aortic area (visualized part): are masked by gases.
- Urinary bladder: is suboptimally distended with urine hindering proper pelvic examination.
- No fluid collection.

CONCLUSION:

- Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.
- GB med with thickened wall.
- Bilateral grade I nephropathy.
- Right renal simple cyst.

Much obliged,

Dr, Mayada Adel



مركز النور المحمدي للأشعة

Alnour Almohammady Radiological Center

مركز الأشعة - مسجد النور المحمدي - مولات صوفية - دوار ملون
22502599 - 22517341 - 26509782 : ت
Alnour Almohammady Mosque - Mesalla Square Tel.: 26509782 - 22517341 - 225502599

PATIENT'S NAME: Ahmad Maatouq

DATE: 15/7/2026

CODE: 2

C.T REPORT

MULTISLICE C.T SCAN OF WAS THROUGH THE ABDOMEN AND PELVIC WITH ORAL AND I.V. CONTRAST INJECTION

- Gall bladder sizable irregular dense stone is noted measuring about 2 cm with relative distended gall bladder and irregular wall thickening more evident along its fundus with marked stranded peri cholecystic fat planes extending to para colic fat along right colonic flexure that also seen diffuse thickened with stranded nodular reaction of surrounding fat planes
- The hepatic surfaces are rather lobular with slightly heterogeneous parenchyma mainly at the gall bladder bed.
- The spleen is of average size with grossly intact parenchyma.
- Right renal sizable cortical cyst is noted measuring about 12x10 cm otherwise, both kidneys are of average size with normal parenchymal thickness and no obstructive uropathy could be detected.
- Good contrast concentration and excretion of both kidneys.
- Normal appearance of the pancreas, aorta, IVC and both adrenal glands.
- Normal CT appearance of the urinary bladder with no stones, masses or diverticular changes
- No ascites or lymphadenopathy could be detected.

IMPRESSION:

- Gall bladder stones are noted with marked irregular mural soft tissue thickening extending to para colic aspect as well as right colonic flexure mural thickening as described above suggestive of ?? inflammatory reaction for clinical correlation and follow up.
- Right renal sizable simple benign looking cortical cyst.

YOURS SINCERELY
DR. MOHAMED FOUAD
PROF. DR OF RADIODIAGNOSIS
DR/ MAHMUD EL AWADY, MSC
AL-AZHAR UNIVERSITY

جمهورية مصر العربية
بمطابقة تحقيق الشخصية

احمد
معتوق على علره
١٣ ش مسجد الرحمن
القنوص - القنوية

١٩٥٦/٠١
K00069371

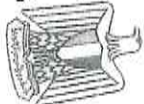
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يون عل

٢٠٢٤/٠٣
معتوق ج

١١
البطاقة سارية حتى ٢٠٣١/٠٣/٢٣



Pathology Report

Clinical Data: Hepatic focal lesion in a patient with gall bladder irregular mural thickening

Nature of specimen: Hepatic focal lesion, Referred block & slide (Tru-cut biopsy).

Gross Examination:

One referred block & one referred (H&E) stained slide labelled by (26H044) and patient Arabic name.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for; CK7 clone (SP52) & CK20 (SP33).

Microscopic Examination

Histopathological examination reveals cores of tumor formed of malignant epithelial cells displaying eosinophilic cytoplasm & enlarged moderately pleomorphic nuclei, arranged in trabeculae & fused glands with single dispersed cells amid desmoplastic stroma. There is focal necrosis.

**** IPT (Control show appropriate reactivity):**

- CK7: Positive in the malignant cells,
- CK20: Negative.

Diagnosis

Hepatic focal lesion, Referred block & slide (Tru-cut biopsy):

- Moderately differentiated adenocarcinoma (Positive CK7).
- Differential diagnosis includes; Upper gastro-intestinal tract (GIT) versus biliary tract origin, as well as other (CK7 Positive) tumors.
- Case for clinical & radiological data correlation.

Signature _____

Prof. Dr. Maha Guimei, M.D., Ph.D.



Name : Ahmed Maatouq Ali
Date : Wednesday, August 19, 2026

pelvi-abdominal U/S reveals

- Liver: is average in size with homogenous parenchymal echopattern. No intra-hepatic biliary radicle dilatation. Left lobe shows well defined iso to hypochoic focal lesion measuring about 2.8 x 2.1 cm, taking no internal vascularity on applying color Doppler.
- PV & CBD (both are patent and not dilated).
- Gall bladder: is average in size shows biliary sludge with increased wall thickness (6mm). no stones
- Both kidneys: are within average size and normal site and shape. Normal parenchymal thickness with preserved cortico-medullary differentiation, yet shows increased parenchymal echogenicity bilaterally. No stones or back pressure changes on either side. Right kidney shows simple cortical cyst measuring about 12.6 x 8 cm.
- Rt kidney: 8.7 x 4.6 x 1.9 cm
- Lt kidney: 10.9 x 5 x 1.3 cm
- Spleen: is average in size with homogenous texture and no focal lesions. Splenic vein is not dilated.
- Pancreas and para-aortic area (visualized part): are masked by gases.
- Urinary bladder: is suboptimally distended with urine hindering proper pelvic examination.
- No fluid collection.

CONCLUSION:

- Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.
- GB mud with thickened wall.
- Bilateral grade I nephropathy.
- Right renal simple cyst.

*Much obliged,
Dr. Mayada Adel*

The General Organization For teaching Hospital
And institutes
Ahmed Maher Teaching Hospital
Advanced Gastrointestinal Endoscopy Unit



الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمي
وحده مناظير الجهاز الهضمي المتقدمه

Colonoscopy Report

ID: 22412
Age: 70

Name: احمد مغتوق علي
Date: Sunday 30, August 2026

Indications

Screening for malignancy.

Medications

Midazolam and Nalbuphine

Instrument

Olympus GIF 160

Procedure Technique

After informed consent was obtained and a physical exam was performed, the patient was connected to the monitoring device and put in the left lateral position. Colonoscope introduced in to the rectum and advanced under direct visualization. The scope was subsequently removed slowly while examining the color, texture, anatomy, and integrity of the mucosa on the way out. The patient was subsequently transferred to the recovery area in satisfactory condition. The following findings were noted:

Findings

Boston preparation (7/9)
Colonoscopy up to the terminal ileum were done and shows no masses or polyps nor ulcers apart from three internal piles grade II-III have been seen on retroflexion.

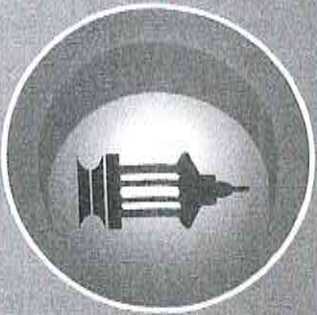
Conclusion

Internal piles.

Recommendation

Upper endoscopy is recommended.

Endoscopist : Dr Omar Azzam MD



مركز النور الرضحي للاشعة

Alnour Almoshary Radiological Center

نايل مسططيسي مفتوح - اشعة مقطعية - موجات صوتية - دوبر ملون
مسجد النور المحمدي - ميدان المسلة
22502599 - 22517341 - 26509782
Alnour Almoshary Mosque - Mesalla Square
Tel: 26509782 - 22517341 - 225502599

PATIENT'S NAME: Ahmad Maatouq

DATE: 15/7/2026

CODE: 2

C.T REPORT

MULTISLICE C.T SCAN OF WAS THROUGH THE ABDOMEN AND PELVIC WITH

ORAL AND I.V. CONTRAST INJECTION

- Gall bladder sizable irregular dense stone is noted measuring about 2 cm with relative distended gall bladder and irregular wall thickening more evident along its fundus with marked stranded peri cholecystic fat planes extending to para colic fat along right colonic flexure that also seen diffuse thickened with **stranded nodular reaction of surrounding fat planes**
- The hepatic surfaces are rather lobular with slightly heterogeneous parenchyma mainly at the gall bladder bed.
- The spleen is of average size with grossly intact parenchyma.
- Right renal sizable cortical cyst is noted measuring about 12x10 cm otherwise, both kidneys are of average size with normal parenchymal thickness and no obstructive uropathy could be detected.
- Good contrast concentration and excretion of both kidneys.
- Normal appearance of the pancreas, aorta, IVC and both adrenal glands.
- Normal CT appearance of the urinary bladder with no stones, masses or diverticular changes
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IMPRESSION:

- Gall bladder stones are noted with marked irregular mural soft tissue thickening extending to para colic aspect as well as right colonic flexure mural thickening as described above suggestive of ?? inflammatory reaction for clinical correlation and follow up.
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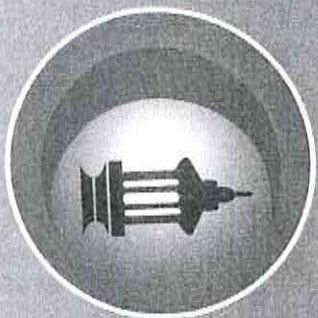
YOURS SINCERELY

DR. MOHAMED FOUAD

PROF. DR OF RADIOLOGICALS

DR/ MAHMOD EL AWADY, MSC

AL-AZHAR UNIVERSITY



مركز النور المحمدي الشعبة

Alnour Almohammady Radiological Center

ثلاثين مقفليسي مفتوح - آفة مقطعية - موجات صوتية - دوبر ملن
 مسجد النور المحمدي - ميدان السيد
 22502599 - 22517341 - 26509782 8 9
 Alnour Almohammady Mosque - Mesalla Square
 Tel: 26509782 - 22517341 - 22502599

Dear collage:

Re: MR: Ahmed Maatouq

Thank you for reference.

The above mentioned patient has significantly distended gall bladder with mural thickening and surrounding soft tissue thickening of the fat planes including the paracolic fat planes along the right colic flexure.

Hypodense hepatic areas of low density is also noted at the gall bladder bed with small focal hepatic lesions.

The central intrahepatic biliary radicles are also slightly dilated.

The right hypochondrium region has been also encroached upon by large simple renal cyst (about 12x10cm).

The appearances might be attributed to (? Chronic inflammatory reaction) (? Infiltrative lesion) for clinical correlation and follow up triphasic study guided by the ultrasound guided biopsy pathological result.

YOURS SINCERELY

DR. MOHAMED FOUAD

PROF. DR OF RADIOLOGNOSIS
 AL-AZHAR UNIVERSITY

جهاز أمن الدولة
مطابقة تحقيق الشخصية



احمد
معتوق علي علره
١٣ ش مسجد الرحمن
القصور - القنوية

١٩٥٦/٠

K00069371

٢٥٦٠١٢٥٠١٠١١٣٧



ذكر

مسلم

متزوج

البطاقة سارية حتى ٢٠٣١/٠٣/٢٣

٢٠٢٤/٠٣ ٢٥٦٠١٢٥٠١٠١١٣٧
يون عل



GUSTAVE
ROUSSY
HISTOPATHOLOGY

Patient Name	احمد مطفى على	Case No.	384721
Sex/Age	Female 70 Y	Patient No.	27751
Registered Date	15/9/2026 09:35:05 AM	Referred By Dr.	Out Patient
Validation Date	19/9/2026 11:36:02 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 10:16:00 AM	Pathology No.	SR-1033/26

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Prof. Dr. Maha Guimei, M.D, PhD.



Name : Ahmed Maatoug Ali

Date : Wednesday, August 19, 2026

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- No fluid collection.

CONCLUSION:

- Left hepatic focal lesion for Triphasic C1 abdomen and pelvis.
- GB mud with thickened wall.
- Bilateral grade I nephropathy.
- Right renal simple cyst.

Much obliged,

Dr. Mayada Jalef

Colonoscopy Report

Name: احمد مطلق علي

ID: 22412

Date: Sunday 30, August 2026

Age: 70

Indications

Screening for malignancy.

Medications

Midazolam and Nalbuphine

Instrument

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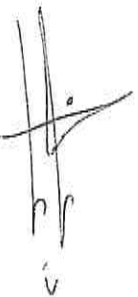
Conclusion

Internal piles.

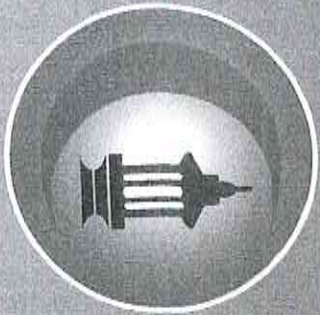
Recommendation

Upper endoscopy is recommended.

Endoscopist: Dr Omar Azzam MD



٠١٢٠٢١٢٤٦٧٨ ت ٨٢٣٩١٤٤٥٥ فاكس القاهرة الدور الثالث فاكس ٣٤١ ش بورسعيد باب الخلق



مركز النور الرضحي للاشعة

Alnour Almoshary Radiological Center

نايل مسطاطيسى مفتوح - اشعة مقطعية - موجات صوتية - دوبر ملون
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PATIENT'S NAME: Ahmad Maatouq

DATE: 15/7/2026

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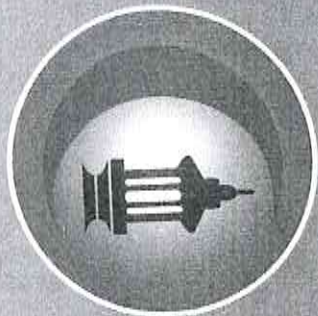
YOURS SINCERELY

DR. MOHAMED FOUAD

PROF. DR OF RADIOLOGICALS

DR/ MAHMOD EL AWADY, MSC

AL-AZHAR UNIVERSITY



مركز النور الإسلامي الشعبة

Alnour Almohammady Radiological Center

ثلاثين مقفليسي مفتوح - آية مقطعية - موجات صوتية - دوبر ملين
مسجد النور (الحمدى) - ميدان السيد
22502599 - 22517341 - 26509782 8 9
Alnour Almohammady Mosque - Mesalla Square
Tel: 26509782 - 22517341 - 22502599

Dear collage:

Re: MR: Ahmed Maatouq

Thank you for reference.

The above mentioned patient has significantly distended gall bladder with mural thickening and surrounding soft tissue thickening of the fat planes including the paracolic fat planes along the right colic flexure.

Hypodense hepatic areas of low density is also noted at the gall bladder bed with small focal hepatic lesions.

The central intrahepatic biliary radicles are also slightly dilated.

The right hypochondrium region has been also encroached upon by large simple renal cyst (about 12x10cm).

The appearances might be attributed to (? Chronic inflammatory reaction) (? Infiltrative lesion) for clinical correlation and follow up triphasic study guided by the ultrasound guided biopsy pathological result.

YOURS SINCERELY

DR. MOHAMED FOUAD

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AL-AZHAR UNIVERSITY